

Patient name: Mary Malloy
Medical Record Number: 123456789
Date admitted: July 1, 2050
Date discharged: July 3, 2050
Attending Physician: Dr. Will Teachwell
Resident Physician: Dr. Bea Goodoc
Diagnosis: Left intertrochanteric hip fracture
Procedure: Left hemiarthroplasty
Orthopedist: Dr. Cal Seeyum
Other diagnoses: Post-op atrial fibrillation
Congestive heart failure – EF of 40% on echo May 2049
Coronary artery disease
Stage 2 Hypertension
Osteoarthritis
Osteoporosis

History:

Mrs. Malloy is a 83 year old woman who presented after slipping on the ice while trying to retrieve her morning newspaper. She landed on her left side and had immediate left leg pain upon trying to rise. Her neighbor noted her fall and called 911 when it was apparent she had been injured. She denied any head trauma or loss of consciousness and no other injuries were found on full body assessment.

Problems:

1. Left intertrochanteric femur fracture – Patient underwent a left hemiarthroplasty on hospital day 1. (Please see full Op note if further details desired.) Her post-p course was complicated by a two day period of atrial fibrillation and volume overload as detailed below, but otherwise she tolerated the procedure well. She started physical therapy POD #1 and has been progressing as expected. She will be discharged to the TLC Rehab Center to complete her course of rehab with the goal of returning to independent living. Her pain is currently well controlled on scheduled Tylenol as below with morphine sulfate 30mg tablet given 30 minutes prior to her morning and afternoon therapy sessions. She will remain on lovenox BID for DVT prophylaxis until she completes her therapy and is once again fully mobile. The surgical site should be evaluated daily for signs of infection, though currently this has not been a

problem. If she continues to heal well, the staples need to be removed on July 12. She is scheduled for follow-up with ortho A clinic as below.

2. Cardiovascular – At baseline she has fair exercise tolerance and has done well on her CHF regimen. POD #1 she developed atrial fibrillation with rapid ventricular response and a medicine consult was called. They adjusted her metoprolol dose and diuresed her with a resolution of her hypervolemia and A-fib in 2 days. Due to her CHF and A-fib she was ruled out for a peri-op MI with serial EKGs and enzymes. TSH was within normal limits. She is currently doing well and is considered to be euvolemic with a weight of 146 lbs.

3. Osteoporosis – Mrs. Malloy has never received a DEXA scan, but was taking vitamin D and calcium supplementation prior to admission, which was increased while here in keeping with new guidelines. She has been referred for ongoing treatment of her osteoporosis and is scheduled to have a DEXA scan on August 1 in the Bone and Mineral Metabolism clinic. We have deferred decisions re: bisphosphonate therapy choices to their expertise.

4. Rehab/Functional Status – At baseline, Mrs. Malloy lived independently and was independent in all ADLs. Her daughter assisted her with grocery shopping and transportation to events out of her immediate neighborhood as she has felt less and less comfortable driving longer distances. Her MMSE was 28/30 on this admission and she participated well in PT and OT – please see attached therapy progress notes for specifics. It is expected she will need 2 more weeks of inpatient therapy to be able to safely return home. At that time, we recommend a home safety evaluation by OT scheduled at a time her daughter can also be present to assist in making her home less of a fall risk.

Medications:

New medications:

Lovenox 30mg SC every 12 hours

Morphine sulfate 30 mg PO twice daily, to be administered 30 minutes prior to her therapy sessions

Tylenol 500 mg 2 tabs PO every 8 hours

Colace 100 mg PO BID

Medications discontinued:

None

Medications continued with dose changes:

Metoprolol 100 mg PO BID

Calcium citrate with vitamin D – 500mg/400IU one tab PO BID

Medications continued unchanged:

ASA 81 mg PO qd

Lisinopril 20 mg PO qd

Furosemide 40 mg PO BID

Spironolactone 50 mg PO qd

Pending Results: None

Follow up: Dr. Orthopedist – July 12 at 3:30 pm

Dr. Primary Care – July 28 at 10:00 am

Dr. Bone Mineral – August 1 at 9:00 am

Cc: Dr. Primary Care – fax 123-456-7890

Dr. Bone Mineral – fax 123-098-7654